

Suicide or Personal Harm Risk

CCG: C-2006 (CCG)

MCG Health
Chronic Care
28th Edition

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Assessment

Q1. Patient: Right now, do you have thoughts of suicide or harming yourself or someone else? **QM**

- Yes [B1] [P1]
- No
- Unknown [B1] [P1]

Q2. Patient: Have you had any of the following symptoms over the past 2 weeks? (Check all that apply.)

- Becoming frustrated or losing your temper easily [P2]
- Crying spells for no reason [P2]
- Eating too much or not wanting to eat at all [P2]
- Feeling guilty [P2]
- Feeling sad or unhappy a lot [P2]
- Feeling tired, sluggish, or physically drained [P2]
- Feeling worthless or hopeless [P2]
- Having trouble sleeping or sleeping too much [P2]
- Little or no interest in activities or hobbies that you enjoy, including sex [P2]
- Little or no interest in being with your friends and family [P2]
- Poor personal hygiene, not wanting to bathe, or not caring about what you wear [P2]
- Thoughts of death, suicide, or harming yourself or someone else [P2]
- Trouble concentrating or remembering things [P2]
- Trouble making decisions [P2]
- None
- Unknown [P2]

Q3. Patient: Have you ever tried to commit suicide? **QM**

- Yes [P2]
- No
- Unknown

Quality Measures

- Disease Management Quality Measures:
 - Information provided from this question may be requested by various accrediting organizations.

Plan of Care

Barriers

B1. Personal harm risk, immediate

Problems

P1. Immediate personal harm (ie, suicide or harm to others) risk present

- **Goal:** Immediate behavioral health evaluation and intervention to prevent patient harming self or others
 - **Interventions:**
 - **ASSIST: If patient is actively threatening harm to self or others, contact the crisis hotline or emergency services while maintaining communication with the patient.**(1) Harming another person can be physical, emotional, or sexual.
(2)

- **ASSIST:** National Suicide Prevention Lifeline: 1-800-273-8255 or call, text, or live chat 988
- **COORDINATE:** Follow-up contact after behavioral health evaluation to review self-care plan and identify need for further assistance(3)(4)

P2. Potential personal harm (ie, suicide or harm to others) risk present

- **Goal:** Personal harm (ie, suicide or harm to others) warning signs and action plan better understood
 - **Interventions:**
 - **ASSIST: If patient is actively threatening harm to self or others, contact the crisis hotline or emergency services while maintaining communication with the patient.**(1) Harming another person can be physical, emotional, or sexual. [A](2)
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of behavioral health status
 - **ASSIST:** Identifying patient supports and setting up support system communication(1)
 - **ASSIST:** Creation of an emergency plan so family and caregivers know steps to take if personal harm symptoms present(1)
 - **ASSIST:** Identifying risks of suicide or harm to self or others(5)(6)(7):
 - Self or family history of suicide or suicide attempts
 - Recent stressful event (eg, recent exposure to another's suicide) or serious adverse childhood event
 - History of being bullied
 - Social isolation or withdrawal
 - Severe functional impairments
 - Spousal loss
 - Discrimination experience (eg, lesbian, gay, bisexual, transgender)
 - Demographic factors (eg, male, older age, non-Hispanic White or American Indian/Alaska Native race)
 - Mental health disorders
 - Triggers include physical pain, loss of support or respect from family or peers, interpersonal conflicts, painful memories, loneliness, helplessness, lack of self-worth.
 - **EDUCATE:** Behavioral health crisis care(5)(8):
 - Crisis care plan: turn off distractions (eg, TV, radio), seek emergent care, call hotline, call police if escalates; caregivers advised to not argue(9)
 - Call police if crisis involves violence.(9)
 - Keep the crisis hotline telephone number near the phone.
 - Keep list of names, addresses, and emergency telephone numbers for medical help and support individuals (eg, neighbor, relative, close friend).
 - **EDUCATE:** Suicide warning signs(10)(11):
 - Depression (eg, feeling sad, hopeless, overwhelmed)(12)
 - Giving away personal possessions, settling accounts, or other actions relating to disposition of estate in case of death
 - Mood instability (eg, extreme or rapid changes)
 - Reckless activities
 - Sense of feeling trapped with no way out
 - Sleep pattern changes (eg, hypersomnia, insomnia)
 - Substance misuse (eg, increased use, use interferes with ADL/IADL)
 - Suicidal or homicidal actions or plans(9)
 - Social changes (eg, isolation, withdrawal)
 - **EDUCATE:** Suicide prevention measures(13):
 - Take any hint of suicide seriously, and seek immediate professional help.
 - Encourage immediate intervention (eg, patient call crisis line, caregiver call provider).(11)
 - Keep list of names, addresses, and emergency telephone numbers for medical help and support individuals (eg, neighbor, relative, close friend).
 - Restrict access to methods of suicide.[B](11)
 - **EDUCATE:** Suicidal ideation signs(11):
 - Expressing suicidal thoughts or actions, or potential for harm to self or others
 - Disclosing of suicide plan or plan to harm another
 - Warning signs for suicide attempt (eg, writing suicide note, giving away valued possessions)
 - Role playing (eg, standing on chair with noose)
 - Recurrent thoughts of death (not just fear of dying)
 - Available means to execute plan of harm to self or others
 - **EDUCATE: When to get emergency help(11):**
 - Suicidal or homicidal actions or plans

- Psychotic behavior (eg, threatening)
- Crisis plan not working(14)
- **EDUCATE: When to call the provider(11)(15):**
 - Depression (eg, feeling sad, hopeless, overwhelmed)(11)(12)
 - Self-harm (eg, abuse, mutilation)
 - Hallucinations or delusions increase.
 - Cognitive changes (eg, thinking problems, forgetfulness worsens)
- **EDUCATE:** National Suicide Prevention Lifeline: 1-800-273-8255 or call, text, or live chat 988
- **COORDINATE:** Behavioral health services referral, if indicated(1)(11)
- **COORDINATE:** Psychiatrist referral, if indicated(1)(11)(16)
- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Protective services or legal authorities, as needed
- **COORDINATE:** Social worker referral, as needed(17)
- **COORDINATE:** Care team meeting, if needed(18)(19)
- **COORDINATE:** Communication between care team members and care plan reconciliation(18)(20)(21)
- **COORDINATE:** Follow-up contact to evaluate safety, referral status, and need for further assistance(3)(4)

References

1. Depression in Adults: Treatment and Management. NICE Guidance NG222 [Internet] National Institute for Health and Care Excellence. 2022 Jun Accessed at: <https://www.nice.org.uk/guidance>. [created 2022; accessed 2023 Aug 08] [Context Link 1, 2, 3, 4, 5, 6, 7]
2. Bipolar and related disorders. In: Morgan KI, Townsend MC, editors. Davis Advantage for Psychiatric Mental Health Nursing. 10th ed. Philadelphia, PA: F.A. Davis; 2021:519-546. [Context Link 1, 2]
3. Care coordination and care transitions to support adherence. In: Perez R, Rogers S, Prince M, Fraser K, editors. Case Management Adherence Guidelines. 2020 ed. Case Management Society of America; 2022:43-56. [Context Link 1, 2]
4. Monitoring and evaluation. In: Gillingham DC, editor. Foundations of Case Management. Blue Bayou Press; 2021:51-52. [Context Link 1, 2]
5. Suicide prevention. In: Morgan KI, Townsend MC, editors. Davis Advantage for Psychiatric Mental Health Nursing. 10th ed. Philadelphia, PA: F.A. Davis; 2021:263-289. [Context Link 1, 2, 3]
6. Lyness JM. Psychiatric disorders in medical practice. In: Goldman L, Schafer AI, editors. Goldman-Cecil Medicine. 26th ed. Elsevier; 2020:2305-2315 e2. [Context Link 1]
7. Reus VI. Psychiatric disorders. In: Loscalzo J, Fauci A, Kasper D, Hauser S, Longo D, Jameson JL, editors. Harrison's Principles of Internal Medicine. 21st ed. McGraw Hill Education; 2022:3540-3556. [Context Link 1]
8. Young HW, Shapiro MA. Suicidal Behavior. In: Walls RM, editor. Rosen's Emergency Medicine: Concepts and Clinical Practice. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:1358-1365.e4. [Context Link 1, 2]
9. Fosbre CD. Suicidal thoughts and behaviors. In: Fosbre CD, Varcacolis EM, Chiappetta L, editors. Varcacolis' Essentials of Psychiatric-Mental Health Nursing. 5th ed. Elsevier; 2023:377-387. [Context Link 1, 2, 3]
10. Steele D. Variables affecting the therapeutic environment: violence and suicide. In: Steele D, editor. Keltner's Psychiatric Nursing. 9th ed. Elsevier; 2023:228-241. [Context Link 1, 2]
11. Mood disorders and suicide. In: Videbeck SL, editor. Psychiatric-Mental Health Nursing. 9th ed. Wolters Kluwer; 2023:284-327. [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10]
12. Depressive disorders. In: Morgan KI, Townsend MC, editors. Davis Advantage for Psychiatric Mental Health Nursing. 10th ed. Philadelphia, PA: F.A. Davis; 2021:479-518. [Context Link 1, 2]
13. Worthington JJ III, Gold AK, Kinrys G. Approach to the patient with depression. In: Goroll AH, Mulley AG Jr., editors. Primary Care Medicine: Office Evaluation and Management of the Adult Patient. 8th ed. Philadelphia, PA: Wolters Kluwer; 2021:1707-1726. [Context Link 1]
14. Psychosocial theories and therapy. In: Videbeck SL, editor. Psychiatric-Mental Health Nursing. 9th ed. Wolters Kluwer; 2023:41-63. [Context Link 1]
15. Major depression. In: Swearingen PL, Wright JD, editors. All-in-One Nursing Care Planning Resource. 5th ed. St. Louis, MO: Elsevier; 2019:744-752. [Context Link 1]
16. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760. [Context Link 1]
17. McQueen A, et al. Social needs, chronic conditions, and health care utilization among Medicaid beneficiaries. Population Health Management 2021;24(6):681-690. DOI: 10.1089/pop.2021.0065. [Context Link 1] View abstract...
18. Case management standards and professional organizations. In: Powell SK, Tahan H, editors. Case Management a Practical Guide for Education and Practice. 4th ed. Philadelphia, PA: Wolters Kluwer, Lippincott, Williams & Wilkins; 2019:314-354. [Context Link 1, 2]
19. Ralston JD, Wagner EH. Comprehensive chronic disease management. In: Goldman L, Schafer AI, editors. Goldman-Cecil Medicine. 26th ed. Elsevier; 2020:42-45.e2. [Context Link 1]
20. Karam M, et al. Nursing care coordination for patients with complex needs in primary healthcare: a scoping review. International Journal of Integrated Care 2021;21(1):16. DOI: 10.5334/ijic.5518. [Context Link 1] View abstract...

21. Joo JY, Liu MF. The experience of chronic illness transitional care: a qualitative systematic review. *Clinical Nursing Research* 2022;31(2):163-173. DOI: 10.1177/10547738211056166. [Context Link 1] View abstract...
22. Suicide Prevention: Optimising Medicines and Reducing Access to Medicines as a Means of Suicide. NICE Key Therapeutic Topic KTT24 [Internet] National Institute for Health and Care Excellence. 2019 Sep [NICE reviewed 2022] Accessed at: <https://www.nice.org.uk/guidance>. [created 2019; accessed 2023 Aug 08] [Context Link 1]
23. Okolie C, et al. Means restriction for the prevention of suicide by jumping. *Cochrane Database of Systematic Reviews* 2020, Issue 2. Art. No.: CD013543. DOI: 10.1002/14651858.CD013543. [Context Link 1] View abstract...

Footnotes

[A] Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Inpatient treatment may be appropriate for people with depression who are at significant risk of suicide, self-harm, or self-neglect.(1) Other patients who will reliably participate in monitoring and have supports and providers available may be safely treated with frequent monitoring in outpatient care.(5)(8) [A in Context Link 1]

[B] Restricting access to suicide methods includes removing firearms or other weapons from the home, keeping only immediate medication dosages available, removing materials that could be used for hanging or suffocation, and access restrictions for jumping (eg, closed-caption video monitoring, physical barrier to bridges).(10)(11)(22)(23) [B in Context Link 1]

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